

Your Health Coverage Should Be Yours.

Not your boss's. Not the government's. Yours.

Right now your health insurance belongs to your job. Lose the job, lose the coverage — usually right when you can't afford to buy it back. And nobody will tell you what anything costs before you get it. You can price-shop a plumber or a plane ticket. Try asking a hospital what an MRI costs.

Here's the idea: instead of your boss owning your health insurance, **you** own it. A policy in your name. A savings account with your money in it. And a guarantee that if you get seriously sick or fall on hard times, you're covered. All three follow you everywhere — every job, every move, every layoff.

TODAY	UNDER THIS PLAN
You lose your job — you lose your insurance. COBRA wants \$2,000/month right when your paycheck stopped.	You lose your job — nothing happens. The policy is yours. And because your income dropped, you automatically get MORE help, not less.
Nobody will tell you what anything costs until the bill shows up.	Prices are posted. For planned care you get a real quote up front — the way your mechanic has to.
The same MRI is \$400 at a clinic and \$2,400 at the hospital across the street.	Clinics can actually open and compete. If hospitals in your area still won't lower prices, price caps kick in automatically.
Skip the insulin because money's tight. End up in the ER.	Insulin, inhalers, blood-pressure pills, screenings, therapy — free from day one. No deductible.

How it works — three simple pieces

1. Insurance for the disasters. Everybody gets a real policy from a private insurance company — competing companies, like car insurance. But they can't turn you away for being sick, or charge you more for it. It covers the big stuff: the cancer, the car wreck, the surgery. There's a deductible, but it's sized to what you actually earn. **And the medicine that keeps you out of the hospital is free from day one** — because a skipped \$40 refill turns into a \$20,000 emergency room bill anyway. Free insulin isn't charity. It's cheaper.

2. An account for everyday care. You get a savings account for routine visits — the strep test, the X-ray, the checkup. Money goes in tax-free, and if you don't earn much, the government puts money in it for you. **It's yours forever, like a 401(k).** You never lose it. And nobody is ever asked to shop around during a heart attack — emergencies and serious illness are handled by piece one, where the insurance company negotiates for you.

3. A safety net that can't be quietly gutted. If you're struggling, or sick enough that you blow past your deductible every year, there's a fund that covers you — and the money is locked in by law, with a minimum floor. Congress can't starve it by just not funding it. **If they want to cut care for sick and poor people, they have to take a public vote to do it.**

Wait — who pays for this?

Almost nobody pays anything new. Here's the part most people have never been told: **your employer already spends around \$18,000 a year on your family's coverage — and that is your money.** It's part of what they pay to employ you. You just never see it, because it goes straight to the insurance company instead of your paycheck. Economists on the left and right agree: if your boss didn't buy your insurance, your wages would be higher by roughly that amount.

So under this plan, your employer hands you that money as wages instead — **and the law requires it.** Same money. But now you can see it, and the coverage is yours.

How we'd actually get there: start with the kids

Nobody trusts a politician who says they'll change health care for 300 million people overnight. So we don't. We cover every child in America first — kids are cheap to insure, and nobody thinks a six-year-old deserves to be sick. Then each year the covered group ages up: the kids become teenagers, become young adults, and it grows from there. No single terrifying vote. Seniors on Medicare are left completely alone.

The honest part

You should hear the weak spots from us, not from an attack ad. If hospitals lobby hard enough to keep competition out, this moves money around without bringing prices down — that's why the automatic price caps are in there, and that fight is real. People who are sick every year don't benefit much from the shopping part, so everything depends on that safety-net fund being genuinely generous. And every past attempt to touch health benefits has died in Congress.

Universal coverage. Your name on the policy. Prices you can see. A safety net that can't be quietly cut.

The American Health Ownership Plan — full framework and cost model available on request